

HHS Health Sector AI RFI

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Submitted by:

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Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (Federal Register Vol. 90, Number 244, December 23, 2025)

Medvidi, Inc. welcomes the opportunity to provide input on this important Request for Information issued by the Department of Health and Human Services (HHS), in collaboration with the Assistant Secretary for Technology Policy (ASTP) and the Office of the National Coordinator for Health Information Technology (ONC).

As a telemedicine provider exclusively focused on behavioral and mental health care, Medvidi has built and actively deploys a suite of native, proprietary artificial intelligence tools deeply integrated into our clinical platform.

Our AI stack includes:

- **Ambient Chart Generation AI:** listens to visits in real time and utilises intake data (with explicit patient consent) and produces structured, clinically dense draft notes that capture key details with high fidelity. Ambient Chart Generation AI provides incremental updates, either adding new content to notes or replacing existing content, every minute so that medical practitioners aren't forced to wait until the end of the visit to see the results of generation.
- **Medical Chart Review AI:** scans every chart post-visit for compliance gaps, missing documentation elements, protocol deviations, and quality indicators, providing feedback to clinicians after each visit.
- **AI Receptionist and Modular Agents:** handle administrative, non-clinical automated patient communications, routine follow-up messaging, adherence prompts, and scalable operational support, all under strict human oversight.

These tools have demonstrably reduced administrative burden (often reclaiming 10–15 minutes per visit previously spent on documentation), improved note accuracy and completeness, strengthened compliance posture, and helped mitigate provider burnout in a field facing severe workforce shortages (with projections estimating a deficit of over 21,000 psychiatrists by 2030).

We offer the following detailed responses to the RFI questions, grounded in our day-to-day experience operating at scale in the telehealth environment.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

From our perspective as an active deployer of clinical AI, several interconnected barriers stand out:

- **Misaligned payment incentives:** Current fee-for-service structures reward visit length rather than efficiency or quality. Tools that dramatically reduce documentation time or enable more consistent follow-up do not generate proportional revenue under traditional billing, discouraging broader adoption even when they clearly improve care delivery and provider well-being.
- **Regulatory uncertainty for non-device AI:** Ambient documentation, chart auditing, and workflow agents occupy an ambiguous space, not traditional SaMD (software as a medical device) but still carrying potential liability exposure in audits, malpractice claims, or enforcement actions. The lack of clear, proportionate guidance creates hesitation among providers and platforms.
- **Evaluation and trust gaps:** Without standardized benchmarks for behavioral health AI performance (e.g., accuracy in capturing emotional nuance or avoiding cultural bias), it is hard to build confidence or compare solutions.
- **Complex web of regulators with differences in priorities:** Behavioural health is regulated by federal and state agencies, including DEA, CMS, SAMHSA, HHS OCR, state licensing boards, and law enforcement bodies, each enforcing overlapping but not fully aligned rules on prescribing, privacy, reimbursement, and professional conduct. With FDA approvals necessary for CDS systems, navigating this complex web of regulatory bodies is not only complicated but potentially unresolved in the current legal framework.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?

We recommend the following prioritized actions, each of which would accelerate responsible adoption based on our internal results:

- **CMS payment modernization:** Introduce reimbursement mechanisms (e.g., add-on codes, modifiers, or value-based incentives) that explicitly recognize time savings and outcome improvements from AI-augmented services, ambient documentation, real-time compliance review, or automated adherence support. When efficiency translates directly to financial benefit, adoption surges.
- **Proportionate, clear guidance for non-device AI:** Publish targeted HHS/ONC/ASTP guidance distinguishing low-risk assistive tools (requiring transparency, consent, and human review) from higher-risk diagnostic/predictive software. This reduces uncertainty without imposing unnecessary pre-market burdens.
- **Establish a cross-agency AI in behavioural health committee.** Given the distinct nature of the agencies that regulate behavioural health, a cross-agency body (including the DEA and FDA) focused on resolving barriers to AI innovation would significantly accelerate unified decision-making across multiple regulatory landscapes.
- **Behavioral health-focused interoperability enhancements:** Expand the United States Core Data for Interoperability (USCDI) to better include mental health elements such as visit transcripts,

PHQ/GAD scores, treatment rationales, and longitudinal outcomes. Mandate secure, standardized APIs to enable benchmarking and federated learning while preserving privacy.

- **Innovation pilots and funding:** Launch grant programs, cooperative agreements, or challenge competitions specifically for mental health AI in shortage areas. These could test real-world impact on access, outcomes, and cost, with public reporting of findings. MEDvidi would be eager to participate and contribute anonymized insights from our dataset.

3–5. For non-medical-device AI tools: What novel issues arise? What evaluation methods seem most promising? What should HHS's role be in supporting private-sector evaluation activities?

Novel issues

- Consent and transparency for ambient audio processing (e.g., granular disclosure, handling of bystanders, easy opt-out).
- Liability allocation in multi-vendor ecosystems (who is responsible if an AI-drafted note contributes to an oversight?).
- Heightened privacy risks with conversational data in mental health contexts.
- Subtle biases in language models that may misinterpret cultural idioms, emotional tone, or non-verbal cues.
- Security and privacy attack vectors (such as prompt injection), which are already managed using guardrails (automated systems that review AI before and after it is invoked).
- The non-deterministic nature of AI means that different results can be obtained for the same input. Strategies like averaging across multiple tries or evaluations help manage that.
- Workflow disruption if tools require excessive clinician editing or produce inconsistent outputs.

Promising evaluation methods

- Pre- and post-deployment comparative studies measuring note accuracy (vs. gold-standard manual drafts), time saved, patient satisfaction, and clinical outcomes.
- Continuous real-world monitoring dashboards tracking error rates, human escalation frequency, bias indicators across demographics, and provider-reported usability.
- Simulation-based testing under realistic conditions (accents, background noise, interrupted sessions).
- Longitudinal tracking of downstream effects (e.g., improved follow-up rates, medication adherence, or reduced no-shows due to less admin burden).
- Using a separate instance of AI to evaluate (LLM as a Judge) the outputs of the original AI system for compliance with a specified guideline, rule, or condition.
- Human annotation of AI outputs feeds back into improvement in how AI is set up.
- Evaluating alignment between independent AI output and human output.

HHS role

- Provide seed funding or cooperative agreements for independent third-party validations, especially for behavioral health applications.
- Endorse voluntary industry accreditation or certification frameworks.
- Organize prize competitions or collaborative challenges to establish benchmarks for specialty-specific AI performance.
- Facilitate de-identified data-sharing consortia to support robust, diverse model training.

6. Where have AI tools in clinical care met or exceeded performance and cost expectations, and where have they fallen short? What novel AI tools have the greatest potential?

Exceeded expectations

Our ambient Chart Generation AI routinely produces notes that are more complete and chronologically accurate than manual drafts, with clinicians editing in under two minutes instead of 10–15. Chart Review AI flags compliance and quality issues at 100% scale, impossible manually, helping maintain standards without added overhead. Chart Review AI doesn't replace human oversight and supervision, which is performed to the full degree necessary by relevant compliance requirements, but provides an additional layer of protection. These have yielded measurable time savings (hours of work per day in case of Chart Generation AI), fewer documentation deficiencies, and improved provider satisfaction.

Fallen short

Generic foundation models without behavioral health fine-tuning often miss sarcasm, contextual history, or cultural expressions of distress, producing bland or inaccurate outputs.

Given that AI systems require continuous improvement and caution should be exercised, Chart Review AI can be overly restrictive in its judgments, resulting in false positives (raising a red flag where a clinical committee doesn't see one).

Greatest potential

Modular, supervised AI agents for: routine follow-ups and dosage titration; real-time evidence-based suggestion during visits; proactive compliance auditing; and scalable virtual support in high-shortage regions. In mental health, these could expand access dramatically while preserving human judgment for complex cases.

7–8. Who are the key players influencing adoption? What interoperability opportunities exist?

Key influencers

- Front-line clinicians (who experience burnout relief firsthand).
- Compliance, quality, and risk management teams (needing defensible audit trails).
- C-suite and finance leaders (focused on ROI and scalability).
- Operations staff (integrating tools into daily workflows).
- Patients (indirectly, through demand for faster/more attentive care).

Interoperability opportunities

Standardized APIs for ingesting visit audio/transcripts into AI pipelines, returning reviewed notes to EHRs, and sharing aggregate outcomes for benchmarking. Federated learning frameworks would allow collaborative model improvement without centralizing sensitive data.

9. What challenges and concerns do patients and caregivers have regarding AI in clinical care?

Patients value faster access, reduced wait times, and providers who appear more engaged. Many appreciate AI freeing clinicians from typing. Primary concerns include:

- Privacy of recorded conversations.
- Fear that AI could diminish empathy or replace human judgment.
- Potential for bias disadvantaging certain cultural or linguistic groups.

We mitigate these through clear upfront consent, visible opt-out mechanisms, explicit statements that clinicians retain final decision-making authority, and transparent explanations of how tools are used.

10. What specific AI research areas should HHS prioritize?

- Multimodal models combining audio, text, validated scales, and outcomes for improved behavioral health support.
- Implementation science examining AI integration into telehealth without disrupting therapeutic relationships.
- Cost-effectiveness analyses in value-based care settings.
- Equity and bias research ensuring performance across demographics, languages, and socioeconomic groups.
- Public-private pilots generating real-world evidence in shortage areas.
- Patient risk modelling for adverse side effects and drug abuse potential

MEDvidi is prepared to collaborate with HHS—sharing de-identified insights, participating in pilots, or contributing to standards development. Our experience shows that thoughtfully designed, specialty-specific AI can meaningfully improve access and quality in mental health care. MEDvidi plans to build AI Clinician capable of diagnosing, treating, and prescribing medications in the behavioural health domain, and is prepared to move jointly with HHS to provide mental health coverage to all people in the US.

Thank you for considering these views. We look forward to continued dialogue.

Sincerely,
Vasili Razhnou
Chief Executive Officer
Medvidi, Inc.